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2020 McQueen Boulevard
Orangeburg, SC 29118

September 27, 2016

Division of Dockets Management (HFA-305)
Food and Drug Administration
5630 Fishers Lane, Rm. 1061
Rockville, MD 20852

Re: Docket No. FDA-2015-D-3719 for "Draft Guidances Relating to the Regulation of Human Cells, Tissues, and Cellular and Tissue-Based Products; Rescheduling of Public Hearing; Request for Comments"

Dear Sir or Madam:

My name is Marie Louise Gehling, and I am a Nurse Practitioner and a certified wound ostomy continence nurse at the Regional Medical Center in Orangeburg, South Carolina. I have been a registered nurse for 32 years; I founded the Wound Center at the RMC in 1992 after seeing too many lower extremity amputations in diabetic patients and those with vascular disease and after taking care of many patients suffering from the stress of having a chronic wound with little available resources.

Wound care has changed enormously since the early 90's, we as providers now have many more resources to manage patients with chronic wounds. We have a viable limb salvage program at our hospital because of advancements in both science and technology; and therefore far fewer lower extremity amputations than in the past.

These advances in science and technology have led to an explosion of growth in the wound care industry; many times this growth has been at the expense of true scientific evidence. One area of ongoing concern has been about the lack of rigorous evidence supporting the therapeutic claims for a growing number of allograft-derived products that are promoted as wound healing agents.

The claims of wound *healing*, reduction in inflammation and reduction in scarring are made by various product manufacturers. These products have been brought to market under Section 361 of the Public Health and Service Act, which only requires controls for mitigating risk of transmission of infectious disease – when additional concerns of safety and efficacy are not raised. In order to be promoted for wound healing the product would have to have gone through the much more rigorous process of PMA/BLA approval. Despite this, manufacturers are marketing products under the section 361 pathway without any FDA pre-market review and then making claims that are not supported by FDA trials as is required under the pre-market approval process.

The current reimbursement by the Centers for Medicare Services increases the confusion about proper use and provides a good example of the confusion created as a result of a lack of regulatory clarity for industry around the meaning of homologous use and minimal manipulation. When CMS bundled the payments for tissue products, this allowed products with FDA reviewed clinical trials to be lumped in with products that have limited actual level one evidence. As a result of this reimbursement methodology change, products without FDA reviewed safety and efficacy data adopted claims from products approved through the pre-market pathway; such as “we are just like product A”— the company who had correctly achieved FDA pre-market approval. This is very problematic if providers are not educated on the differences in the standards for products regulated under Section 351 vs 361.

When manufacturers realized they could get payment by making such claims without FDA approval, the market place for Human cellular products and tissue products erupted and continues to expand exponentially. The unregulated growth in the industry -- not supported by valid scientific evidence or rigorous research -- has taken guidance documents that are clear in their verbiage and bastardized them to meet their own needs, thus leading to false or misleading claims of wound *healing* – for which FDA has defined very specific criteria. If a manufacturer wants to cite a therapeutic claim for healing or reducing inflammation, then it must be supported in rigorous human trials. The allowance of payment for a product does not lend legitimacy to its claims, however it does lend to

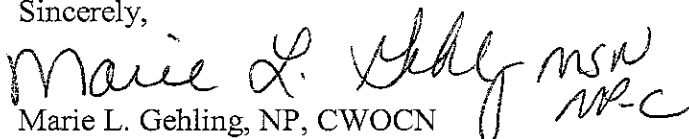
confusion. Why would CMS allow for the same payment of product that is not an FDA approved therapy, when there are safe and proven therapies backed by sound, FDA-reviewed evidence and that meet FDA's high standards for safety, efficacy and quality.

Our patients are entitled to the highest quality of care available. They are entitled to know that their care providers are not being misled as to the nature of, or risks associated with, the therapeutic products they are receiving. They are entitled to rely upon the natural assumption that we all make, that someone -- other than the companies who stand to profit from the sale of a particular therapy -- has reviewed the data to support the therapeutic claims for the product and has determined that those data are robust, are derived from well-designed and well-executed clinical trials, and are relevant to the particular therapeutic claims being made.

Unfortunately, today patients are being lied to by omission. The system is failing them. Because in fact many of the products being marketed as advanced wound healing biotherapies have never been reviewed on a premarket basis; neither is there any oversight of their claims on a post-market basis. And in fact, there is virtually no adverse event reporting for these products. To make matters worse, this reality is beginning to play out in other therapeutic areas as well. For these reasons, I urge FDA to finalize the guidance documents under discussion today with all possible haste, in order to strengthen the boundaries between products properly regulated solely under Section 361 and Section 351.

I thank you for the opportunity to comment on these very important guidance documents.

Sincerely,

The handwritten signature of Marie L. Gehling is written in dark ink. It is a cursive script that includes the first letters of the first and last names, with a stylized 'G' and 'H'. To the right of the signature, the letters 'msw' and 'NP-C' are written in a smaller, more legible hand.

Marie L. Gehling, NP, CWOCN